Three OAGB Revision Propositions Compared with Prof. Wei-Jei Lee's Baseline Plan

Scope: written opinions for a patient with OAGB (2017), chronic bleeding marginal ulcer, bile reflux, Hill-IV hiatal hernia, gallstones, iron deficiency and unconfirmed *H. pylori* eradication.

Compared and scored: Dr Usama Iqbal · Dr Murat Üstün · Dr Fakhrul Islam. **Baseline only:** Prof. Wei-Jei Lee.

Evidence review completed 8 November 2026 · Source documents: the four email-correspondence .docx files in this repository

Clinical safety notice. This is an educational comparison of written proposals, not a medical diagnosis, treatment recommendation, or assessment of any surgeon's skill. A chronic bleeding ulcer and recent anaemia require direct review by a qualified bariatric/upper-GI team. Scores assess the *proposal as documented in email*, not outcomes, ability, hospital quality, or the suitability of a surgeon for this individual.

1. What the literature supports — and what remains uncertain

- **RYGB conversion is a well-supported direction for medically refractory bile reflux after OAGB.** A 2024 systematic review/meta-analysis of 134 reported conversions found reflux was the commonest indication and near-complete symptom improvement, but the evidence base was small, observational and heterogeneous. Major postoperative complications were pooled at 9.4%. This supports conversion as a serious corrective operation, not a low-risk routine procedure. [1]
- **There is no single established conversion template.** Published approaches vary widely: alimentary limbs 50–150 cm and biliopancreatic limbs 50–200 cm in the 2024 review. A small 12-patient reflux series preserved the gastrojejunostomy and created a jejunojejunostomy 75 cm distal to it, with symptom improvement at one year. Therefore, preserving the old anastomosis is a published approach, but it is not proven superior for a large chronic bleeding ulcer. [1][2]
- **Marginal-ulcer management is multi-factorial.** The ASMBS review addresses ulcers after both OAGB and RYGB; PPI therapy, risk-factor management, endoscopic surveillance and surgery for complicated/refractory disease all remain relevant. The evidence does not support assuming that bile alone explains every ulcer. [3][4]
- ***H. pylori* remains important to settle.** Its exact role in post-bariatric marginal ulcer is still uncertain, but ACG guidance recommends proof of eradication after treatment. Any interruption of acid suppression in this particular patient must be planned by the treating clinicians because of the prior bleed. [4][5]

2. Prof. Lee's proposition — the baseline for comparison (not scored)

Prof. Wei-Jei Lee — Preserve old GJ + divert bile + truncal vagotomy

Written plan: adhesiolysis/anatomy check; hiatal-hernia repair; truncal vagotomy; divide the afferent limb and create a jejunojejunostomy 40 cm below the original GJ (possibly 60 cm); circumferentially suture rather than resect the original GJ; close mesenteric defects. Cholecystectomy only if symptomatic or preferred.

Strengths

- Directly addresses bile flow while avoiding a fresh GJ anastomosis, which may reduce operative dissection and leak exposure.
- Preserving the GJ during conversion has precedent in a small reflux series, although its 75 cm limb was longer than the 40–60 cm discussed here. [2]
- Most specific stated experience for a similar operation (~30 cases over 20 years), but this is self-reported correspondence rather than independently audited data.
- Gives an explicit strategy for hiatal repair and mesenteric-defect closure.

Limitations / questions

- The plan leaves a large chronic previously bleeding ulcer in place, with no planned tissue diagnosis in the written reply. The explanation should be made explicit.
- Truncal vagotomy may be an adjunct for refractory marginal ulcer, but modern reports generally describe it with revision/resection of the GJ; the incremental benefit after bile diversion alone is uncertain. [6]
- Does not present a structured nutrition, anaesthetic, *H. pylori*, or endoscopic follow-up pathway in writing.
- Cholecystectomy is optional despite post-RYGB access challenges; this is a trade-off, not an obvious error, and merits an individual discussion.

3. Comparison and scoring of the three alternatives

Question	Dr Usama Iqbal	Dr Murat Üstün	Dr Fakhrul Islam	Compared with Lee
Core proposition	RYGB conversion; resect the old ulcerated GJ and create a new one.	RYGB conversion or reversal, final intra-operative choice; ulcer may need local revision/resection.	RYGB conversion with a refashioned new anastomosis.	Usama/Fakhrul replace the ulcer-bearing GJ; Murat reserves decision; Lee preserves it.
Bile-reflux solution	Divert bile with RYGB. Limb lengths not stated.	Divert bile with RYGB if appropriate. Limb lengths not stated.	AL 75 cm + BP 50–60 cm stated, but anatomical definitions should be confirmed by the operating surgeon.	All use RYGB diversion. Published limb lengths vary; no email plan establishes an evidence-based “correct” number. [1]
Vagotomy	Not proposed.	Not proposed.	Explicitly opposed as not valid for this case.	Lee adds truncal vagotomy. Evidence supports careful case selection; it is not clearly a required component of OAGB□RYGB conversion. [6]
Hiatus / gallbladder	Repair hiatal hernia and remove gallbladder in same operation.	Repair if confirmed; cholecystectomy considered, not automatic.	At least cruroplasty if indicated; insists on same-sitting cholecystectomy.	Murat is most conditional/individualised; Usama and Fakhrul are most decisive.
Pre-operative pathway	Defined lab set + anaesthesia assessment and teleconsultation.	Most comprehensive written pathway: nutrition, medication reconciliation, anaesthetic/cardiovascular and fit-to-fly assessment; staged fallback.	Not specified; defers intra-operative details to operating surgeon.	Both Usama and Murat are materially more structured than Lee’s email plan.
Written experience disclosure	~30 mixed revisions; 25 hiatal repairs/year; reports no complications.	Does not disclose OAGB□RYGB case volume in writing.	No operating series; advisory opinion only.	Lee reports the most directly relevant experience, though all email figures require verification.

Scoring method. The scores below are not clinical outcome predictions. They grade only the written propositions against: (i) fit with current evidence and the stated clinical problem (35%); (ii) technical specificity and ability to resolve the ulcer/bile issue (25%); (iii) peri-operative safety/optimisation (25%); and (iv) transparency and practical deliverability (15%). Scores are deliberately close because all three share the central evidence-supported approach: RYGB conversion for refractory bile reflux.

Dr Usama Iqbal — 7.5 / 10

Score 7.5

Why this score: The most definitive ulcer strategy as written: it removes the ulcer-bearing GJ, creates a new anastomosis, combines RYGB/hiatal repair/cholecystectomy, and adds a concrete pre-operative work-up. This compares favourably with Lee's preservation strategy where the ulcer has already bled.

Pros

- Resection offers a direct solution to diseased anastomotic tissue and an opportunity for pathology, if performed.
- Clear all-in-one plan with anaesthetic review and nutritional/iron testing.
- Correctly does not make vagotomy an automatic requirement.
- Strongest practical deliverability: named hospital pathway, surgeon, devices and coordination.

Cons / questions to close

- No limb lengths, ulcer pathology plan, leak strategy, or post-operative endoscopy/PPI plan stated in writing.
- Its 30-case figure is for mixed revisions, not a procedure-specific OAGB□RYGB series; "no complications" should be clarified with definitions and timeframe.
- *H. pylori* is deferred post-operatively without a detailed eradication/confirmation plan.
- Automatic cholecystectomy adds operative work; clarify the case-specific balance in an asymptomatic patient.

Dr Murat Üstün — 7.2 / 10

Score 7.2

Why this score: Strongest safety and diagnostic posture. The plan explicitly treats the case as multi-factorial, checks nutrition and anaesthetic risks, recognises a staged alternative, and does not encourage unsupervised PPI withdrawal. It scores lower than Usama because the actual operation is less specified.

Pros

- Only proposition that explicitly says the ulcer should not be assumed to be bile-only.
- Most complete pre-op pathway, including nutrition, medication discrepancy reconciliation and planning around hypersomnia/stimulant therapy.
- Offers a staged fallback if active bleeding, anaemia, inflammation or nutrition make a one-stage procedure unsafe.
- Most cautious written approach to *H. pylori* testing and PPI cessation.

Cons / questions to close

- Does not commit to resect-vs-preserve the GJ, limb lengths, defect closure, or a vagotomy position.
- Volume/outcomes for OAGB revision were asked for but not supplied in writing.
- "Conversion or reversal" is a useful option set, but the decision criteria need to be defined before consent.
- Commercial-package messaging should remain separate from clinical decision-making.

Dr Fakhrul Islam — **6.4 / 10**

Score 6.4

Why this score: A coherent technical advisory opinion that improves on Lee's plan by recommending a new anastomosis and specifying limb lengths, while avoiding routine vagotomy. The score is limited because it is not an operative offer and does not document an assessment, pre-op plan, outcomes, or accountable implementation pathway.

Pros

- Clear prioritisation of anatomical bile diversion and refashioning the GJ, rather than relying on acid reduction alone.
- Gives explicit limb numbers, which makes the proposition discussable with the operating surgeon.
- Recognises the importance of hiatal repair and the practical difficulty of future biliary access after RYGB.
- Correctly identifies that revision surgery should be done in an experienced setting.

Cons / questions to close

- Over-attributes the ulcer to bile reflux; ulcer biology is multi-factorial, and *H. pylori* status still needs a plan. [3][4][5]
- "No vagotomy" is reasonable as a default opinion but should not rule it out categorically in a truly refractory acid-driven ulcer.
- No procedure-specific outcomes, pathology plan, anaesthetic/nutrition pathway or centre of delivery.
- The advice to focus on benefits and avoid extensive risk discussion is not sufficient for informed consent in a complex revision.

4. Bottom line and decision questions

Interpretation

Dr Usama Iqbal's written proposition is the strongest of the three as an immediately executable surgical plan, mainly because it removes the ulcerated GJ and includes pre-operative assessment. **Dr Murat Üstün's is the strongest diagnostic/safety framework**, but it needs a final surgical specification and case-volume disclosure. **Dr Fakhrul Islam's is a useful technical second opinion**, particularly on avoiding automatic vagotomy and on giving limb targets, but it is advisory and cannot be selected as an operative plan without an accountable team.

None should be chosen solely on this document. Lee's preservation-plus-vagotomy method is not invalid — preservation has published precedent — but for a large chronic ulcer with a bleed, the reason to keep rather than resect the ulcer-bearing GJ should be fully explained and independently reviewed.

Questions that should be answered in writing before consent

1. Will the ulcerated GJ be preserved or resected, and why in this specific anatomy? If preserved, what is the plan for biopsy and for non-healing/recurrence?
2. What are the exact alimentary, biliopancreatic and common-channel measurements/targets, and how are they intended to balance bile diversion, weight change and nutrition?
3. Is vagotomy proposed? If yes, why after anatomical bile diversion, and what are the expected risks/management of diarrhoea, stasis and dumping?
4. What is the confirmed *H. pylori* plan, including a safe test-of-cure strategy, rescue regimen/susceptibility if still positive, and coordination with the gastroenterologist?
5. What procedure-specific OAGB□RYGB outcomes can the operating surgeon provide: leaks, bleeding, reoperation, readmission and mortality, with denominator and timeframe?
6. How will iron, ferritin, vitamin D, B12, folate, albumin and weight be monitored after revision? What PPI and endoscopic follow-up schedule is planned?
7. Why cholecystectomy now versus surveillance? Does the surgeon advise a one-stage operation after reviewing current haemoglobin, iron, ulcer status and nutritional markers?

5. Sources and evidence limits

1. Sargsyan N, et al. Outcomes of OAGB conversion to RYGB: systematic review and meta-analysis. Obesity Surgery, 2024. Six observational studies; 134 patients. This is the most directly relevant synthesis, but is limited by small cohorts and heterogeneity.
2. Kermansaravi M, et al. Conversion of OAGB to RYGB in symptomatic reflux without resizing the pouch. Journal of Minimal Access Surgery, 2021. A 12-patient case series, not proof of superiority for preservation or for a specific limb length.
3. ASMBS literature review: treatment of marginal ulcers after metabolic and bariatric surgery, 2024.
4. ASMBS literature review: marginal-ulcer risk factors, screening and prophylaxis, 2024.
5. American College of Gastroenterology: 2024 H. pylori treatment guideline summary.
6. Truncal vagotomy and gastrojejunostomy revision for treatment of marginal ulcer. PubMed record, 2025. Illustrates vagotomy as an adjunct in a refractory-ulcer series; it does not establish it as mandatory in OAGB conversion.
7. Revision/conversion surgeries after OAGB: experts' modified Delphi consensus. Obesity Surgery, 2024.

Primary documents: the repository's four Word files: *Emails Prof. Wei-Jei Lee*; *Emails with Dr. Usama Iqbal Aga Khan University Hospital*; *Emails Istanbul Bariatric Center Team*; and *Opinion of Dr Fakhrul Islam*. The case facts and each proposition are reported from those documents and were not independently verified.

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